Case Study — The Shape-Adaptive Brief

This is a case study about using exploration to de-risk a decision quickly, and about shipping the part that's ready while the rest earns its evidence.

2026-09-20 · @Someone

The short version

The Brief — the pre-visit summary a physician reads in the seconds before walking into the room — was working as a concept and failing as a page. Clinicians were being asked to read where they needed to scan.

I ran a series of design experiments to find out why, studied how mature products in adjacent industries solve the same problem, and turned what came back into three information-architecture improvements that shipped into the alpha build.

The experiments deliberately went further than what shipped. I used them to probe an open architectural question — whether different patient types need structurally different Briefs — and brought it to clinical and product leadership early, before any of it was expensive. The review gave me a clear read on where the boundary sat. I took the parts that fit inside decisions the team had already aligned on, shipped those three weeks later, and left the larger question open with defined signals for when to revisit it.

This is a case study about using exploration to de-risk a decision quickly, and about shipping the part that's ready while the rest earns its evidence.

Context

The product is a clinical assistant used by primary care physicians at the point of care. The users see 40+ patients a day and have no dedicated prep time between them.

The surface in question is the **Brief** — the pre-visit summary a physician reads in the seconds before walking into the room. Its job is catch-up context: what happened to this patient since you last saw them. A separate surface, the **Agenda**, handles what to act on today. That distinction matters later.

My role. Staff designer, and the design lead on the visit experience from its origin.

I led the visioning for the redesign. I facilitated vision workshops with the chief product officer, the design director, and clinical stakeholders, then synthesized that input into a vision prototype for the end-to-end flow. That prototype became the reference for the platform redesign.

From there I worked with product leadership to turn the vision into a roadmap — breaking it into big rocks and milestones, and forming development teams around the individual workflows. I held the architecture while the design systems team built a new design system for the AI-native approach, and the workflow teams pushed their pieces of the vision forward against their own requirements and feasibility constraints.

By mid-summer that had produced fourteen parallel prototypes across several teams. My work shifted to convergence: coalescing those explorations back into a single coherent end-to-end flow, and owning the Brief's information architecture within it. This case study covers that phase.

#	Decision	Aligned by
1	Surface data without editorializing — "like the most humble med student." Trust is lost fast when the system makes clinical interpretations.	Working Group
15	One experience, three data profiles — same UX pattern, different data density.	Product lead, clinical lead, design director

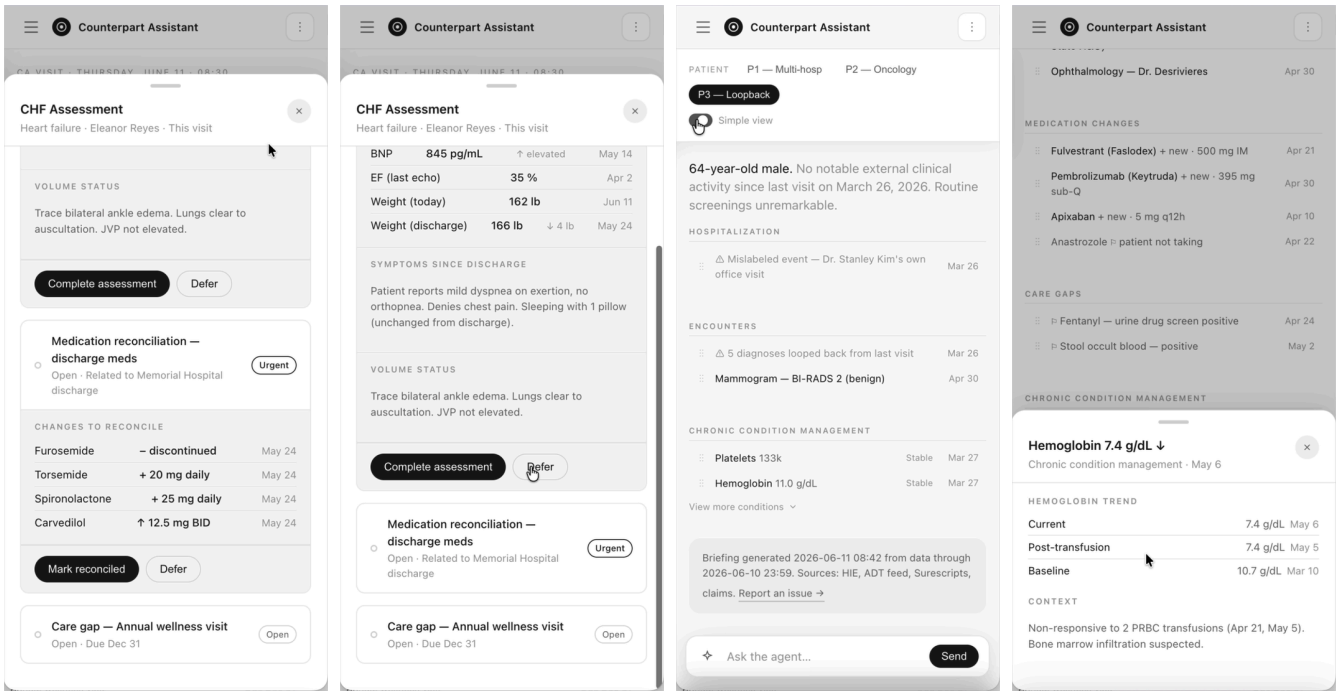
Two standing decisions. By the time the Brief work began, the team had aligned on two principles that shaped what I could propose:

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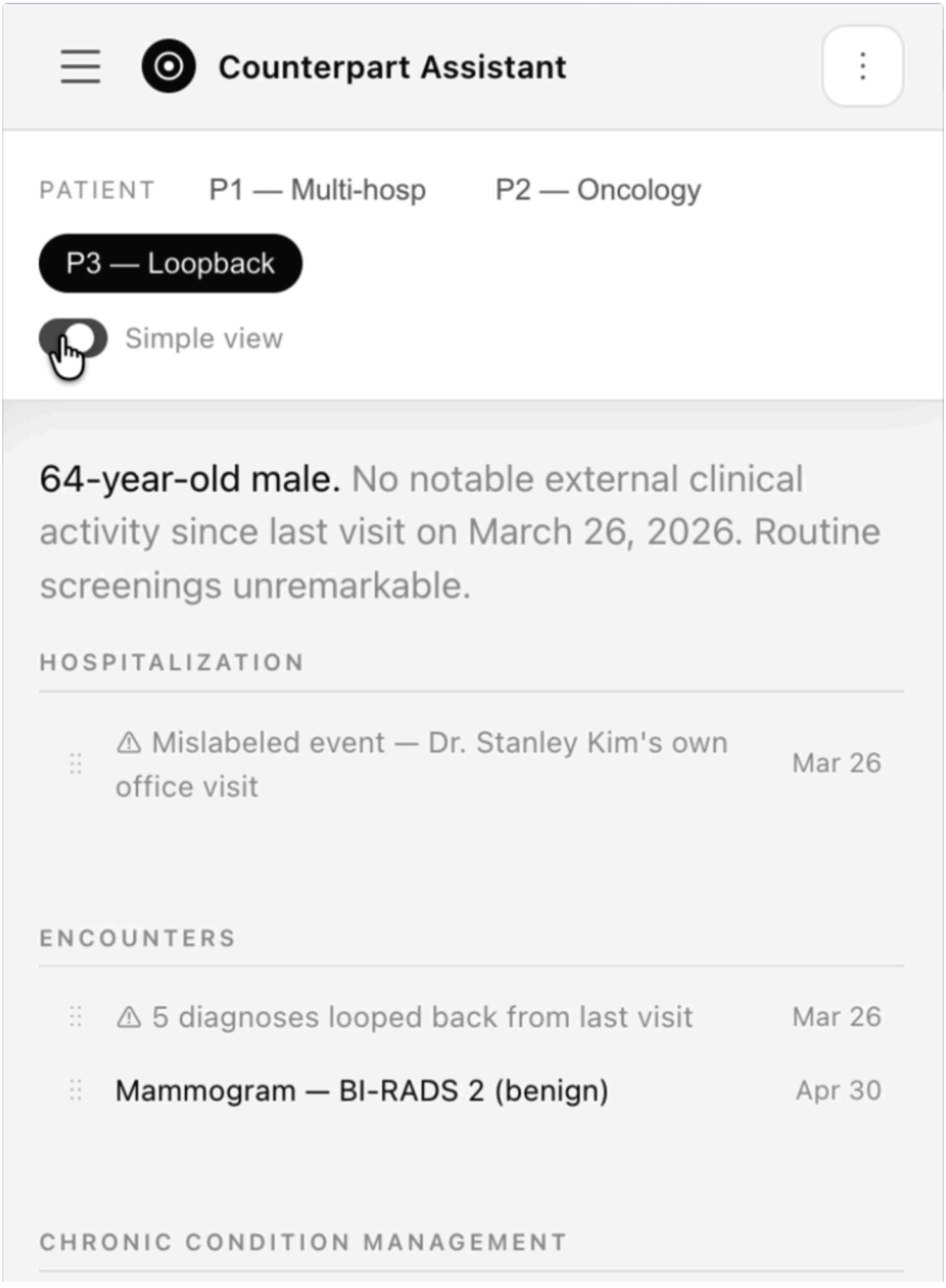
What I'd already established. Before the Brief work, I'd run an exploration of the activity-since-last-visit content — what a physician needs to know about what happened to a patient while they were away. I worked it deliberately in grayscale, unstyled, to keep review conversations on the clinical content rather than the visual treatment.

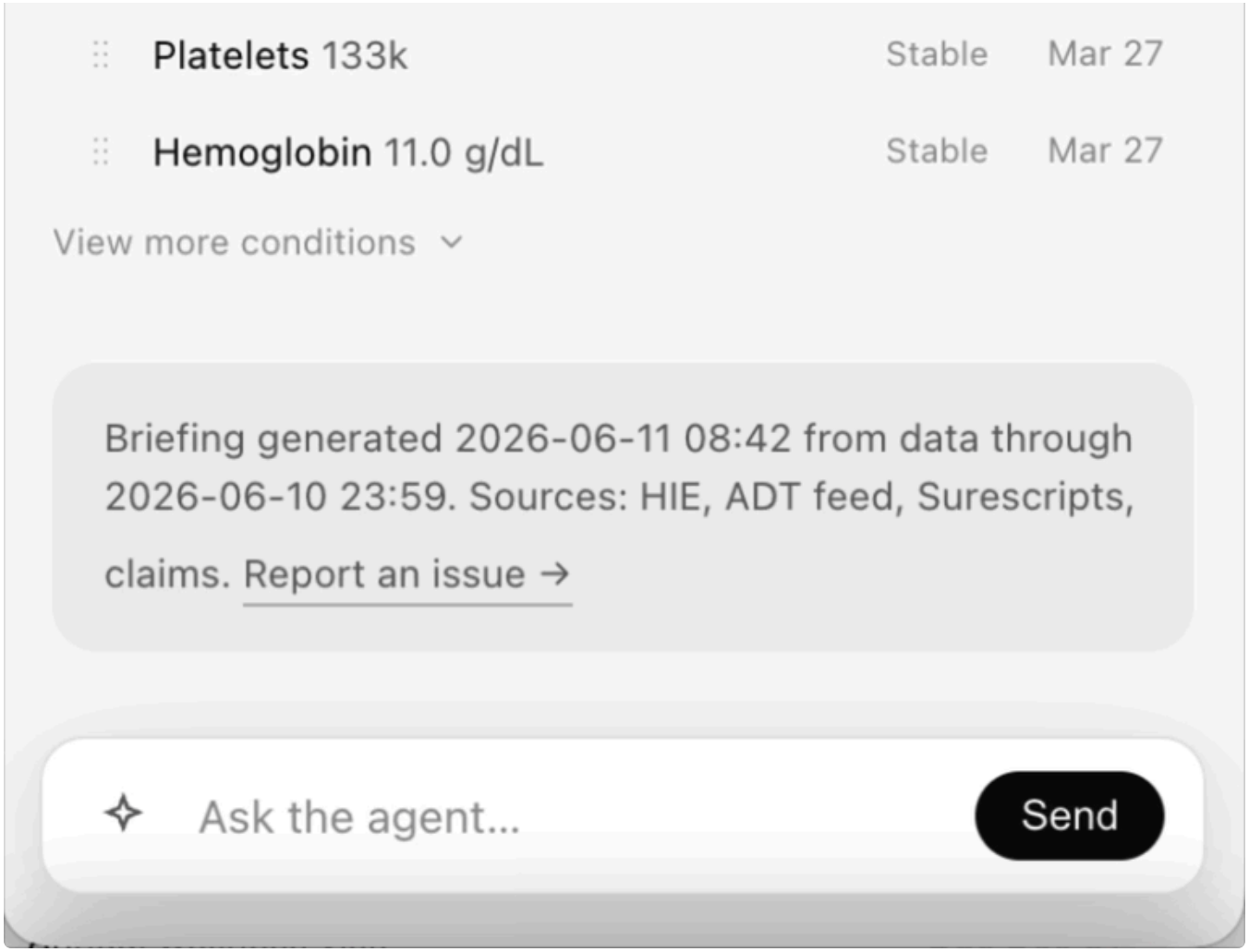
That exploration set three patterns that matter later in this story:

- **An explicit generation window and source list** — *generated at 08:42 from data through 23:59 the previous night; sources: health information exchange, admit-discharge-transfer feed, pharmacy claims* — with a way to report a problem with any of it.
- **Medication reconciliation as a first-class view** — what was discontinued, started, and dose-changed at discharge, presented as a list to reconcile rather than prose to read.
- **Values with their trend and context**, not values alone — a hemoglobin of 7.4 shown against its post-transfusion and baseline readings, with the clinical note that the patient hadn't responded to two transfusions.



The sparse case mattered most. When nothing much had happened, the brief said so in a single line — no notable external clinical activity since the last visit, routine screenings unremarkable — rather than padding to look substantial. Data-quality problems were surfaced too, not hidden: an event mislabeled as a hospitalization when it was the physician's own office visit, and diagnoses looped back from the previous visit, both flagged in place.


Counterpart Assistant
PATIENT
P1 — Multi-hosp
P2 — Oncology
P3 — Loopback
Simple view
64-year-old male. No notable external clinical activity since last visit on March 26, 2026. Routine screenings unremarkable.
HOSPITALIZATION
⚠ Mislabeled event — Dr. Stanley Kim's own office visit
Mar 26
ENCOUNTERS
⚠ 5 diagnoses looped back from last visit
Mar 26
Mammogram — BI-RADS 2 (benign)
Apr 30
CHRONIC CONDITION MANAGEMENT



Leadership's read was that this content was well-researched and validated, and directed that it be merged with another designer's visual treatment for the next iteration. So I entered the Brief work with the content argument already won and the presentation argument still open. That distinction turns out to be the whole story.

The problem

The Brief was working. The prose inside it was not.

Early alpha feedback was genuinely positive on the concept. One physician called the consolidation of outside visits a clear win. Another called the assistant "so good." The Brief was doing the right job.

What failed was presentation. Each section delivered several sentences of continuous prose — a 68-year-old male with uncontrolled diabetes, moderately elevated PSA, and a newly diagnosed foot ulcer, on a background of hypertension, carotid artery disease, and

COPD — where a clinician needed to scan. We had structured the data and then re-flattened it into paragraphs.

A second, sharper complaint: every count on the Brief ("7 specialist visits," "9 medications") prompted the same question. *Since when?* One participant raised it repeatedly across sessions. Once he understood everything was scoped to the last visit, the Brief made sense. The scope was real but implicit.

The observation that started the hypothesis. I laid five real patient briefs side by side and noticed the system was emitting the same template for all of them — for a patient with three uncontrolled conditions, for a patient fresh out of a hospitalization, for a patient whose only news was eight new medications started by five different prescribers, and for a patient with nothing to report.

That reframed the problem for me. The question was not *what should the layout be.* It was *how many kinds of patient does this need to handle, and should each get its own shape?*

Research: looking outside healthcare

Clinical software has no mature convention for this. Every EHR I studied was fighting the same density problem and losing in the same way. So I went looking for industries where the problem was older and better solved.

The reframe: *brief a busy expert on what changed since they last looked* is not a clinical problem. It is a general one. Sales teams solved it for pre-call prep. Operations teams solved it for incident response.

I studied two reference classes:

Domain	Products studied	Why
Sales prep	Gong, Sybill, Granola, Clari, Common Room, Salesforce Einstein	Rep walking into a discovery call has the same job: back-to-back meetings, rich history, needs what changed, fragile trust
Incident response	Datadog, PagerDuty	Severity triage and evidence-linking under time pressure
Clinical analogs	Epic Storyboard, Regard, Abridge, Suki, One Medical, Oscar	Closest problem space, least mature patterns

The headline finding was not the one I expected. Nobody in the mature reference class writes narrative AI summaries. Not one. Every serious product uses templated structure, type-aware sections, and traceable evidence. The two-sentence "AI read of the story" I had been sketching was not a pattern that survives contact with production.

Six patterns recurred across every serious tool:

1. **Templates by meeting type, not one-size-fits-all.** Gong ships eight brief templates. Sybill adapts by meeting type. Granola's entire model is picking the right template first.
2. **Traceability is table stakes.** Every section links to the call, email, or transcript that produced it. Evidence is threaded through, not parked in a footer.
3. **Structure beats prose, consistently.** Even the products that call their output a "brief" deliver a sectioned artifact.
4. **Human writes the structure, AI fills the detail.** Granola's inversion of the usual pattern — the human owns the shape, the AI does the labor.
5. **The brief is paired with an ask-anything surface.** The brief is an entry point, not the answer.
6. **Time-window pickers are standard.** A 30-day snapshot for renewal risk versus a six-month lookback for a QBR. Different jobs, different windows.

Visit Briefing — comparative analysis

Products worth studying for how they turn dense underlying data into a scannable, decision-ready view. Grouped by proximity to our problem.

On the screenshots: most of these products protect their marketing imagery against cross-origin embedding, and Epic / Ironclad / Gong screenshots live behind login walls entirely. Rather than embed brittle image URLs that break, each card links directly to the best public source I found — official product page, App Store listing, or design gallery. Bloomberg Terminal is the exception (CC-licensed on Wikimedia). For a Miro board, open each source link, capture the screenshot yourself, and drop it in — takes ~20 minutes and gives you higher-quality visuals than hotlinks would.

Pattern tags: Hierarchy · Delta / change · Severity · Evidence chips · Since-last · Density craft

DIRECT HEALTHCARE ANALOGS

Epic Storyboard

EHR

Left-rail patient summary in Hyperspace.

SCREENSHOTS: RCHSD tip-sheet PDF · U. Iowa training · eHealth glossary

Note: Epic's own screenshots live behind Epic UserWeb login. Hospital training materials are the best public source.

HIERARCHY Commits to one glanceable vertical column; everything else is a click.

DENSITY CRAFT Chips for allergies, code status, isolation — no prose.

> **Take:** Ambitious hierarchy discipline. Still dense, but the priority order is deliberate.

Regard

CLINICAL AI

AI-suggested conditions with evidence from the chart.

SCREENSHOTS: regard.com · Product press page · PPLE Labs walkthrough

HIERARCHY "Conditions the AI thinks you should address" as the primary frame.

EVIDENCE CHIPS Each suggestion carries the underlying data that supports it (ejection fraction, meds, labs).

> **Take:** Closest problem to ours. The "here's the finding + here's why" pattern is exactly what "Needs attention today" is reaching for.

Abridge

AMBIENT SCRIBE

Post-visit summaries with transcript-linked evidence.

SCREENSHOTS: abridge.com · Play Store (clinician app) · Contrary Research breakdown

EVIDENCE CHIPS Every note element traces back to the transcript excerpt that generated it.

HIERARCHY Sectioned output (HPI, ROS, A&P) resisted re-flattening to prose.

> **Take:** Same "structured extraction, don't re-flatten" tension we're facing. The traceability pattern is especially strong.

Suki

CLINICAL AI

Ambient documentation + chart Q&A.

SCREENSHOTS: suki.ai/clinicians · App Store listing · DeepCura review (screenshots)

SINCE-LAST Explicit "what's changed since last encounter" framing.

DENSITY CRAFT Compact but still prose-y — closer to our current state than we'd want.

> **Take:** Useful as a "not-quite" — shows the failure mode of staying prose-oriented even after structuring.

One Medical

PRIMARY CARE

Member-facing app with visit history + upcoming care.

SCREENSHOTS: App Store listing · Technology page · Virtual care guide

SINCE-LAST Timeline-first framing for care history.

> **Take:** Consumer polish applied to clinical data — good counter to EHR-convention defaults.

Oscar Health

PAYER / CONSUMER

Member timeline + care navigation.

SCREENSHOTS: App Store listing · "How we designed Oscar 2.0" (Medium) · hioscar.com/app

SINCE-LAST Timeline organization vs. category-first.

> **Take:** The Medium design case study is especially useful — historical screens showing the redesign rationale.

ADJACENT DOMAINS (WHERE THE PATTERN IS MORE MATURE)

Datadog — service overview

OBSERVABILITY

On-call triage view for engineers.

SCREENSHOTS: Service Page docs · Service Map docs · APM Dashboard guide

SEVERITY Color-coded status is the primary signal; details are secondary.

DELTA / CHANGE Every metric shows change + last-updated-at.

HIERARCHY "What's broken right now" ranked above "what's fine."

> **Take:** The clearest execution of the triage pattern anywhere. Directly transferable to "Needs attention today."

Mercury / Ramp

FINTECH

Account overview cards with value + delta.

SCREENSHOTS: Mercury live demo · Mercury Insights · Ramp Reporting

Mercury's demo dashboard is interactive — great for showing the pattern live.

DELTA / CHANGE Value + delta + sparkline as the atomic unit.

DENSITY CRAFT Restraint — few cards, big numbers, clear hierarchy.

> **Take:** Consumer-grade elegance for numeric summaries. "HbA1c 8.1% ↑ from 7.4" owes a lot to this pattern.

Bloomberg Terminal

FINANCE (PRO)

Extreme density done deliberately.

SCREENSHOTS: Wikipedia article · Wikimedia Commons category

DENSITY CRAFT Every character earns its place; nothing is decorative.

DELTA / CHANGE Color = direction; magnitude = size; time = position. Encoded, not written.

> **Take:** Counterweight to "clinicians need whitespace" — professionals absorb density if it's encoded well.

Linear — cycle view

PM / ENG

Team's current work at a glance.

SCREENSHOTS: Cycles docs · Uiland gallery (251 screens) · SaaSUI gallery

HIERARCHY Groups by status, not by type.

SEVERITY Priority + blocked state as icons, not labels.

> **Take:** Excellent example of grouping by "what state is this in" rather than "what kind of thing is it."

Gong — deal warnings

REVENUE

Pre-meeting deal briefs with risk flags.

SCREENSHOTS: Deal board docs · Deal Warnings PDF guide · Deal Execution page

EVIDENCE CHIPS "Deal warnings" surface risks with source callouts (emails, calls, activity).

SINCE-LAST "What changed since your last touch" as the lead.

> **Take:** Nearest analog outside healthcare — same "prep me before I walk in" job. Care gaps ≈ deal warnings.

Ironclad

LEGAL

Contract review with issues surfaced up top.

SCREENSHOTS: Review Contracts product page · AI review demo video · G2 reviews (user screenshots)

HIERARCHY Open issues at top; clause-by-clause detail below.

EVIDENCE CHIPS Each issue links back to the clause it came from.

> **Take:** Triage-then-detail structure. Same shape as the redesign's "attention today → stable → background."

PagerDuty — incident feed

OPS / ON-CALL

Live severity-sorted incident stream.

SCREENSHOTS: Incidents page docs · Analytics Dashboard docs · PD University walkthrough

SEVERITY Sort order is severity, not time. Red = triggered, orange = ack'd, blue = clear.

SINCE-LAST "New," "acknowledged," "resolved" as visible states.

> **Take:** Rare product where the cost of missing something is life-safety-adjacent — worth studying their conservatism.

Attio — record view

CRM

Configurable summary over messy underlying data.

SCREENSHOTS: SaaSUI (25 screens) · Figma community (250+ screens) · Attio on Dribbble

DENSITY CRAFT Small type, tight grid, big information yield.

> **Take:** Good reference for "how much can we fit before it feels cramped."

CONSUMER HEALTH (ADJACENT, WORTH A GLANCE)

Apple Health — summary

CONSUMER

Daily health snapshot: Pinned + Highlights + Trends.

SCREENSHOTS: Mobbin (Health Summary) · Apple Support guide · App Store listing

HIERARCHY "Highlights" (curated) sits above "Browse" (exhaustive).

> **Take:** Curation-as-hierarchy — the system chooses what matters instead of showing all.

Oura · Whoop

WEARABLES

Single-score summary with driver metrics beneath.

SCREENSHOTS: Oura Readiness (SwipeFile) · Oura blog · Whoop App Store · Whoop Recovery guide

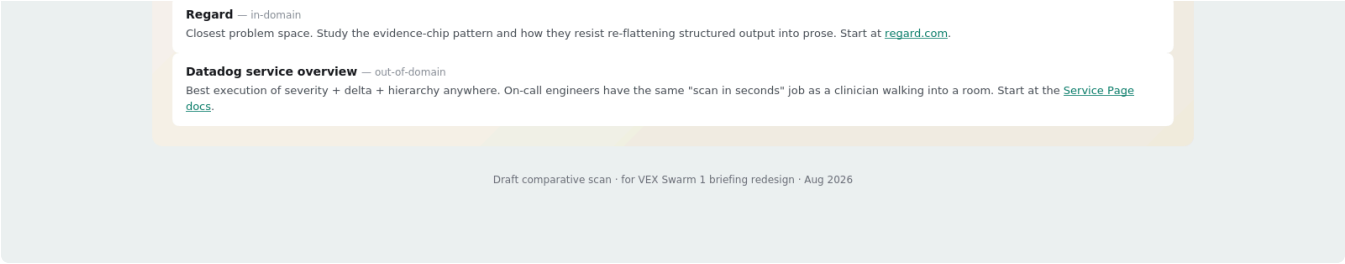
DELTA / CHANGE One big number + trend + drivers underneath.

SEVERITY Whoop's red/yellow/green ring is a masterclass in status-at-a-glance.

> **Take:** "One number, why, trend" template. Also handles the emotional weight of bad news — clinicians face that too.

IF THE POD ONLY STUDIES TWO

The pairing that tends to move the conversation: one in-domain doing it okay, one out-of-domain doing it exceptionally. The contrast loosens up the "but healthcare is different" objection faster than more healthcare examples do.



Pattern 1 looked like direct validation of the shape hypothesis. Pattern 6 was a clean answer to the *since when?* complaint. Pattern 3 killed an idea I had been attached to.

The hypothesis and the five shapes

Stated cleanly: different patient profiles need meaningfully different Brief shapes, and a one-size-fits-all Brief will keep failing at its job even after the information architecture improves.

The sub-claims:

1. Briefs vary along axes that matter clinically — recent inpatient event, medication churn, cross-specialty coverage, single-domain dominance, sparseness. These are not cosmetic differences.
2. A single template will either pad sparse patients, making the summary feel unreliable, or thin out complex ones, missing the actual story.
3. The mature reference class has converged on templated-by-purpose as the right answer.
4. The clinician's cognitive shape when scanning follows the patient, not a fixed structure. A fixed structure asks them to do more mental switching than a shape-matched brief would.

I built five archetypes against real anonymized briefs, same underlying data model, different presentational shape:

Shape	Trigger	What leads
High-acuity	3+ uncontrolled values or new diagnoses	Triage list of abnormal findings
Recent event	Hospitalization or ED visit in lookback	The event anchors everything; discharge meds prominent
Med reconciliation	Medication churn dominates	New starts grouped by prescriber, polypharmacy flag

Shape	Trigger	What leads
Sparse / stable	Low signal	Short and honest — do not invent hierarchy
Single-domain deep	One specialty dominates	That domain's thread, other conditions collapsed

The sparse case was the one I cared most about. If nothing much happened, the Brief should say so in six lines. Padding it to match a complex patient's length teaches clinicians the summary is decorative.

Visit Briefing — five patient shapes

Same underlying data model, five different presentational archetypes. The AI currently emits one shape regardless of patient; this mock shows what it looks like when the shape adapts.

> **The real issue isn't visual — it's architectural.** Looking across five real briefs, the AI is generating one template regardless of whether the patient has 3 uncontrolled conditions, a hospitalization, a med-reconciliation follow-up, or nothing at all. The question isn't "what should the layout be" — it's "what are the 4-5 patient archetypes this needs to handle, and does each get its own shape?"

1 · HIGH-ACUITY

Multiple uncontrolled findings

70M · original example · foot ulcer + PSA + HbA1c

Brief — Last visit Apr 2 · 4 mo

70M · T2DM, HTN, CAD, AAA, COPD

Since last visit: new dx — foot ulcer · 2 new meds · PSA ↑↑

NEEDS ATTENTION TODAY

- **Diabetic foot ulcer** NEW — Podiatry started · doxy + mupirocin ›
- **PSA** — Urology referral? — 53.7, ↑ from 12.4
- **HbA1c** — On metformin — 8.1%, ↑ from 7.4
- **Triglycerides** — HDL 21 — 446, ↑ from 312

2 open care gaps — Review ›

BACKGROUND

CAD · AAA · psoriasis · MCI · anxiety ›

26 dx · 8 meds · 11 labs — AI

When to use: 3+ uncontrolled values or new diagnoses. The triage is the point.

2 · RECENT EVENT

One acute event anchors the brief

78F · hospitalization + AKI

Brief — Last visit Mar 18

78F · T2DM, HTN, hyperlipidemia, COPD, dementia

HOSPITALIZED · MAR 24–25

CareWell Medical Center

Recurrent dizziness, LUE weakness, L carotid stenosis · d/c with med changes

FOLLOW-UP FROM ADMISSION

- **AKI — worsening** — Consider nephrology — Cr 2.02, ↑ from 1.8 (Mar 23)
- **K⁺** — Trending up — 5.5 ↑
- **Ca²⁺** — Workup? — 10.9 ↑

MED CHANGES AT D/C

NEW **Aspirin · saxagliptin · meclizine**

STOP **Cipro · donepezil · levocetirizine · olmesartan**

DOSE **Sertraline ↓ · atorvastatin adj.**

2 open care gaps — Review ›

23 dx · 8 meds — AI

When to use: Hospitalization, ED visit, or new dx within lookback. The event *is* the brief.

3 · MED RECONCILIATION

Med list is the news

85M · resistant HTN + multiple new starts

Brief — Last visit Apr 2

85M · resistant HTN, T2DM w/ circ. complications, HLD

Since last visit: 8 new meds started · 9 refill encounters · *no new labs*

NEW MED STARTS

NEW **Metformin** — T2DM

NEW **Carvedilol** — HTN

NEW
Sacubitril / valsartan
CHF

NEW
Finasteride
BPH

NEW
Albuterol
COPD

NEW
Sertraline · donepezil · clobetasol
depression · dementia · rash

CONSIDER TODAY

Med reconciliation
8 starts across 5 providers · interaction check? ›

1 open care gap — **Review ›**

10 dx · 8 new meds — AI

When to use: Med churn is the dominant signal. Promote what changed and why, flag polypharmacy risk.

4 · SPARSE / STABLE

Not much to report — say so

74F · routine follow-up

Brief — Last visit Jul 6

74F · HTN, HLD, chronic AFib

Since last visit: routine follow-up · *no significant changes*

WATCHING

TSH
Recheck / adjust levothyroxine?
14.0 ↑

Osteoporosis (LS)
Rx review · fall risk
T −4.0 ↓ worsening

RECENT VISITS

Podiatry (foot care) · IM (chronic care) ›

BACKGROUND

HTN · HLD · AFib · PVD ›

9 dx · 1 recent Rx (prednisone) — AI

When to use: Low signal. Be short, honest. Don't invent hierarchy. Length should match what's actually happening.

5 · SINGLE-DOMAIN DEEP

One specialty dominates

64M · 3 psych visits + CKD/DM background

Brief — Last visit Feb 26

64M · T2DM, CKD 3a, HTN, HLD, MCI

Since last visit: 3 psych visits (90d) · UACR trending up

BEHAVIORAL HEALTH

3 visits w/ Alex Mullins · Mar–May
GAD · PTSD · MDD · sedative/hypnotic dependence — same dx pattern across all three; stable on regimen.
See individual notes ›

NEEDS ATTENTION TODAY

HbA1c
Uncontrolled · regimen change?
10.5% ↑

UACR
CKD 3a · ACE/ARB?
93 ↑ elevated

2 open care gaps — **Review ›**

BACKGROUND

HTN · HLD · MCI ›

7 dx · 2 care gaps — AI

When to use: Repeated visits in one specialty. Collapse the pattern, don't list each visit. Let PCP see the story, not the log.

CROSS-CUTTING PRINCIPLES (APPLY TO EVERY SHAPE)

- **Values carry deltas.** A number without its trajectory is a lie — patient 1's "AKI (Cr 1.8, eGFR 29)" hides that Cr is trending 1.8→2.02.
- **Length matches content.** Sparse patients get short briefs. Padding a stable patient to look "thorough" trains clinicians to distrust the summary.
- **Absence of findings is not news.** Collapse "negative ketones, negative glucose, negative bilirubin…" into "UA unremarkable."
- **Round machine-precision numbers.** `73.483 kg` → 73 kg. `1.651 meter` → 1.65 m. Reads as trustworthy, not as raw output.
- **Fix upstream data leaks.** `albumin_creatinine_ratio_unit`, `glomerular_filtration_rate_unit`, and ALL-CAPS provider names all undermine polish on a per-patient basis. Design can't hide these — they need to be fixed at the source.
- **Care gaps hide when zero.** Don't render an empty section; don't put "0" as a metric.
- **Shape selection is a decision the AI has to make** — probably based on: is there a recent inpatient event? are there ≥3 abnormal values with deltas? is med churn high? is one specialty dominating recent visits? If none, use sparse.

I proposed a decision rule: recent inpatient event routes to shape 2, three or more abnormal deltas route to shape 1, and so on.

The leadership review

I took the full exploration — five shapes, the routing logic, the research behind it — to product, clinical, and design leadership while it was still cheap to change. The review did what reviews are for: it surfaced constraints I could not see from inside the design work.

Four things came back, and each one sharpened the proposal.

Predictability has real value at volume. A clinician reading forty Briefs a day builds muscle memory for where things live. Varying structure by patient trades that away. I had weighted richness of insight above speed of retrieval; for this user, at this volume, retrieval wins. That is a user-advocacy argument I had on the wrong side.

The Brief and the Agenda are architecturally distinct. The Brief is catch-up context with a wide net; the Agenda is focused action. Several of my shapes had started to read like action plans — useful work, but belonging to a different surface. The product lead's read was that shape-adaptive thinking might have a real home in Assessments, which opened a separate track rather than closing the idea.

Narrative synthesis was already settled. My own competitive research had found that nobody in the mature reference class ships narrative AI summaries. Leadership arrived at the same conclusion independently. That convergence was useful — it retired the direction cleanly instead of leaving it to be relitigated later.

A standing decision already covered the question. Decision #15 — one experience, three data profiles — had been aligned three weeks before I joined, by people with deeper context on the clinical cohort than I had. Moving it would take evidence from real alpha usage, not a sample of five Briefs.

Engineering added a practical constraint: deterministic structure was cheaper and more predictable to build than signal-driven template selection, which mattered with an alpha date fixed.

The exploration had done its job. In one session it established where the boundary sat, what evidence would move it, and what could ship immediately.

What shipped instead

Three weeks later I brought a narrower proposal. It opened by naming the two standing decisions and stating plainly that it worked within both and was not asking to re-open either.

That sentence did more work than anything in the first deck.

Two of the three had already proven out in my earlier activity-since-last-visit work — the explicit window with its source list, and evidence shown alongside every claim. They were not new proposals. They were patterns leadership had already endorsed in one context, brought into the surface where they were missing.

Three improvements, each traceable to a research finding and each surviving from the original argument:

1. Since-last-visit framing. The header states the window explicitly and persists across sections. Every count is scoped to it, unambiguously. Direct answer to the *since when?* complaint; matches the time-window pattern from sales prep.

2. Traceability inline. Findings carry the data that produced them, threaded through rather than parked in a verify affordance. Consistent with the humble-med-student principle, and it answered trust concerns two clinicians had raised.

3. Structural spine, not narrative. Fixed section structure, scannable content beneath it — the human-writes-structure pattern, adapted. Medication changes get New/Stop/Dose badges at list level. Labs get high/low flags. Both had been uncontested asks from a participant, sitting unbuilt.

And one principle that is a diluted form of the shape argument, which survived precisely because it is a principle rather than an architecture change: **length matches content.** The Brief for a stable patient can be six lines. No padding. That gets a meaningful part of what I wanted without asking anyone to re-open a decision.

Making the Brief scannable.

Three IA improvements to the current Brief. Built on top of the v3xp prototype, aligned with existing design decisions, validated by prior art.

John Blanco-Slingerland · Design · leadership alignment

WHERE WE ARE

The Brief works — the wall of text inside it doesn't.

Cecilia (P2) called the consolidation of outside visits a clear win. Nelson called CA "so good." The Brief is doing the right job.

What's failing is the presentation *inside* each section. Multiple sentences of prose per section forces the clinician to read where they should be able to scan.

Existing decision #1 (Working Group, 2026-06-22): "*Surface data without editorializing — like the most humble med student. Trust is easily lost when the system makes clinical interpretations. Earn it incrementally.*"

Existing decision #15 (Kevin, David, Molly, 2026-06-30): "*One experience, three data profiles — same UX pattern, different data density.*"

This proposal works *within* both of those decisions. We're not asking to re-open either.



1 SINCE-LAST-VISIT FRAMING

Make the time window unmissable.

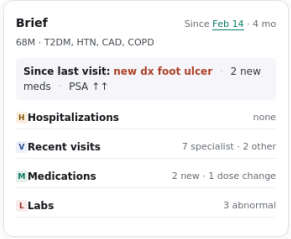
Every count on the Brief today ("7 specialist visits", "9 medications") raises the same question from clinicians: *since when?*

Corey (P3) flagged this repeatedly. Once he understood everything was scoped to "since last visit," the Brief made sense. The problem was that scope was implicit.

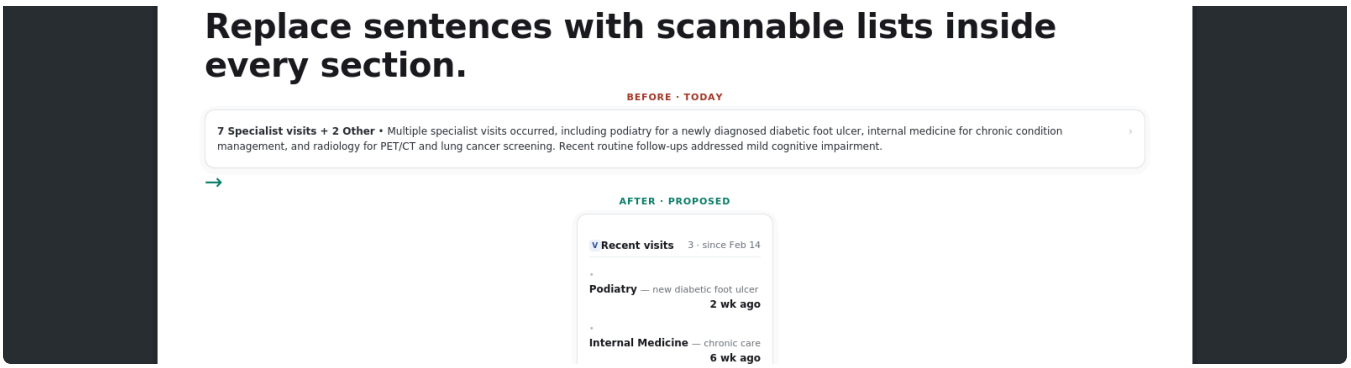
What we're proposing:

- Header explicitly says **"Since [last visit date]"** — persistent across sections
- Optional strip surfaces what's *new* since last visit — new dx, new meds, values trending in wrong direction
- Every count in each section is implicitly scoped to that window; no ambiguity

Prior art: Gong's AI Briefer, Cotera, Sybill all lead with time-window framing. Corey's ask matches an industry pattern.



2 STRUCTURAL SPINE, NOT PROSE



All three shipped into the alpha build.

And a plan for the open question. Rather than drop the architectural thread, I defined what would tell us it needed revisiting, so the team would recognize the signal instead of relying on someone remembering to ask:

- Alpha usage shows clinicians engaging with the Brief differently by patient type
- Post-alpha sessions surface the same structural ask from three or more clinicians, unprompted, *after* the IA improvements have landed
- A specific workflow — post-discharge follow-up being the likeliest — consistently breaks on a single-shape Brief
- A competitor ships something similar and clinicians in our cohort notice

This let the team commit fully to the consistent-structure direction without pretending the question was permanently closed. The bet was reversible, and everyone knew what would reverse it.

Reflections

What shipped: three IA improvements into the alpha build — explicit time-window framing, inline traceability, and a structural spine — plus the length-matches-content principle. Each traces to a specific research finding. Each survived from an exploration that went considerably further.

What I'd carry into the next one.

Socialize an architectural proposal before the decision forum. Walking the clinical lead and medical directors through it individually first would have surfaced prior context earlier — including a related direction the team had already tested — and let the idea arrive with advocates rather than cold.

Separate the easy ask from the hard one. My first deck bundled "make the Brief scannable" with "let the Brief change shape." The first was uncontroversial and ready. Bundling them held a ready improvement behind an open architectural question for three weeks. Splitting them would have delivered user value sooner and given the larger idea a cleaner, less pressured hearing.

Size the evidence to the size of the ask. Five Briefs produced five archetypes — enough to justify an exploration, not enough to justify an architecture. Naming which one I was making, going in, would have set the right expectation for the room and for me.

Treat standing decisions as accumulated context. Decision #15 encoded reasoning from people closer to the clinical cohort than I was. Understanding what it was protecting, before proposing around it, would have told me exactly what evidence could move it.

What I'd do again. The cross-domain research. Going to sales-prep and incident-response tools produced every finding that shipped, and it independently retired my weakest idea before it ever reached a review. Research that changes your own mind is doing its job.

Production notes

Working notes for turning this draft into the published piece — not part of the case study itself.

Anonymization already applied. Company, product, and all individuals are unnamed. Stakeholders appear by role (product lead, clinical lead, design director). Physicians appear as participants or not at all. Decision numbers are kept because they carry the point that these were formal, recorded decisions — swap them for "a standing decision" if even that feels close.

The images need an anonymization pass of their own. The deck renders carry your byline and, more importantly, participant references — one clinician named directly, two others as coded participants — plus the three leadership names attached to the standing decisions. The ASLV frames carry invented patient names and a physician's name inside a flagged data-quality note. All of it needs blurring or cropping before publication, and it is easy to miss because the text sits inside images rather than in the prose.

Tone check applied. The piece now reads as facilitation — running experiments to inform a decision, aligning stakeholders, shipping iteratively — rather than as a disagreement with an outcome. The leadership review is framed by what it clarified. Reflections are forward-looking.

Attribution. The role section now reflects the full arc: visioning, workshops, vision prototype, roadmap, architecture, convergence. The visual explorations in the middle phase were the workflow teams' work — this piece claims the vision, the architecture, the research, the Brief IA, and the convergence, which is accurate and stronger than the narrower version. Worth keeping the distinction visible so a reader knows which parts were yours directly and which you were directing.

Visual assets to build.

Asset	Status
ASLV grayscale exploration, four states	**Placed**
Sparse patient with provenance footer	**Placed**
Five patient shapes	**Placed**
Cross-domain comparative board	**Placed**
Narrow proposal — before/after and framing	**Placed**
Full deck renders and 14 shipped-prototype screens	In the asset folder, not yet placed

Length. Roughly a third shorter now. If it still runs long on the page, the next cut is the five-shape table down to three rows, and the six research patterns down to the three that shipped.

Open question on positioning. For music-tech and AI-tool roles, the transferable claim is the cross-domain research move and the discipline around an AI-generated surface — not the clinical domain. Consider a one-line opener that names that explicitly.